

Analyzing Market Dynamics and Revitalizing Brand Strategy for Market Cannibalization of an Injectable Anesthesia Drug

BIA 810
Health Care Data & Analytics
Final Project

Team 2

Spring 2026

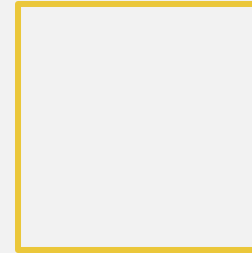
Meet the Team!



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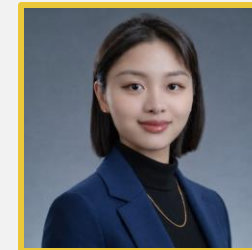
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Today's objectives & agenda



Topic



Presenter

Kick off & Team Introductions

Group 2

Business Problem & Market Share Trends

Puja

Writer Productivity & Territory Drops

Bhuvan

Diagnosis, Specialty, Patient Age Trends & Writer Trends

Aakarsh

Strategic Recommendations

Chetan

Data Gaps, Future Analytics & Conclusion

Hsin-Yu

Overview



- **Executive Summary** — Business problem and competitive landscape of the injectable anesthesia market
- **Market Trends by Product** — Analysis of claims, patient share, and HCP adoption trends across Products 1–4
- **Regional & Specialty Insights** — Territory performance and specialty-level market trends impacting product growth
- **Writer & Market Driver Trends** — Evaluation of prescriber behavior, claims per writer, and patient trends
- **Strategic Recommendations** — Data-driven recommendations to improve Product 2 adoption and regain market share

Executive Summary

Problem Statement

In the injectable anesthesia market, the cannibalization strategy is failing. Product 2 was launched to take over the market share that Product 1 (the market leader) has been steadily losing. Instead, a competitor's brand, Product 3, has captured most of that share — and Product 2 is now also declining, losing claims, patients, and prescribers each year.

Objective

Identify the drivers behind Product 2's underperformance, evaluate the competitive dynamics enabling Product 3's rapid growth, and recommend data-driven actions to recover share in the injectable anesthesia market.

Approach

Analyze claims, patient, prescriber, territory, and specialty trends across 2016–2018 Medicare data to diagnose Product 2's decline and surface targeted, data-driven recommendations.

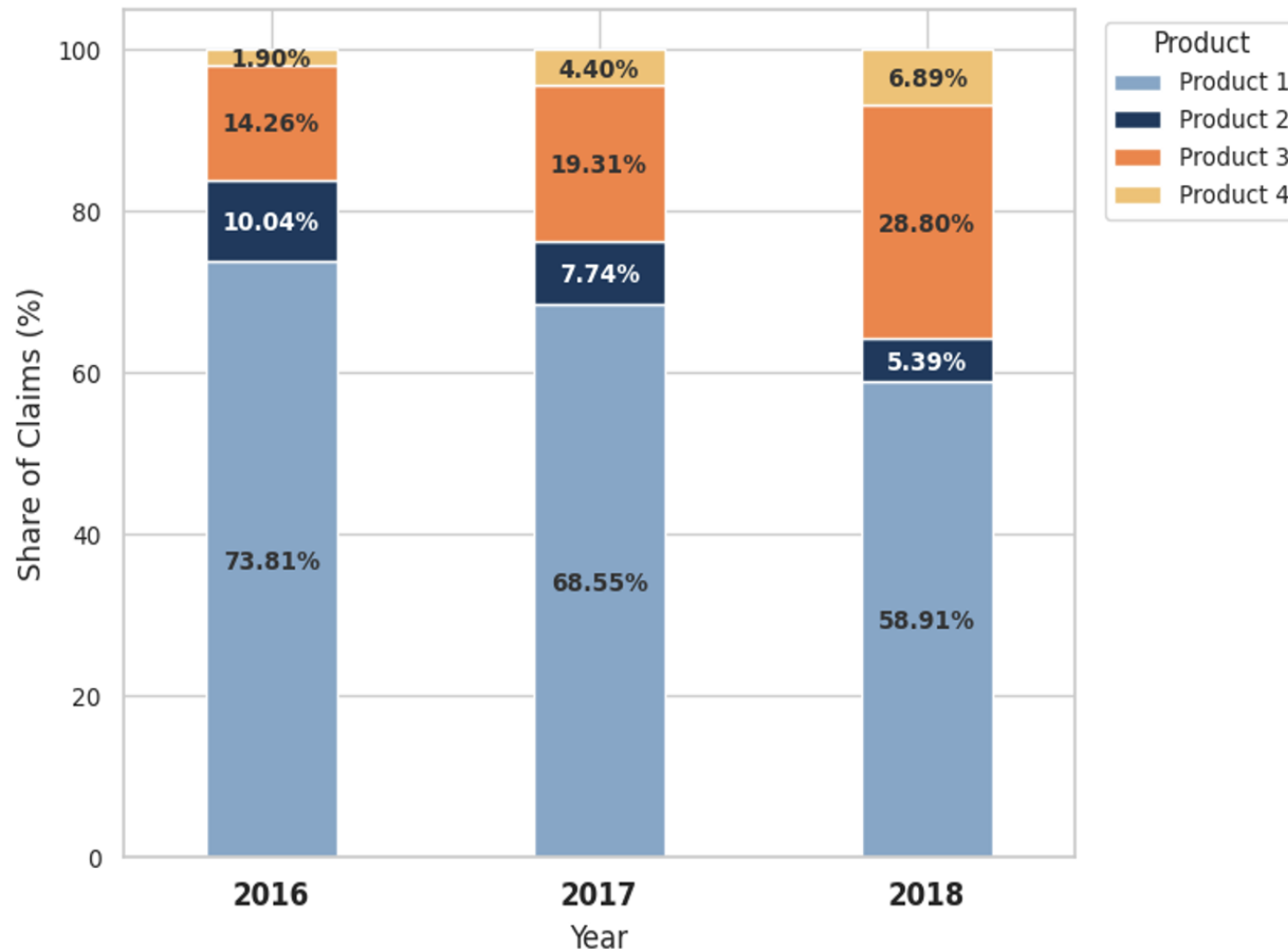


Market Basket

Product	Brand Relationship	Procedure Code	Generic Name	Product Description
Product 1	Market Leader (Original Brand)	J1885	ketorolac tromethamine	Ketorolac is used for the short-term treatment of moderate to severe pain in adults. It is usually used before or after medical procedures or after surgery
Product 2	Variant Brand (New Brand)	J2250	midazolam hydrochloride	Intravenously injected for induction of general anesthesia
Product 3	Main Competitor Brand	J3010	fentanyl citrate	For analgesic action of short duration during the anesthetic periods, premedication, induction, and maintenance, and in the immediate postoperative period (recovery room) as the need arises
Product 4	Alternative Competitor Brand	J2704	propofol	Propofol injection is used to help you relax or sleep before and during surgery or other medical procedures. This medicine is an anesthetic and a sedative

Share of Claims by Product (2016–2018)

Claims volume share in the injectable anesthesia market



-14.9 pts

Product 1 share lost over 3 years (claims)

+14.5 pts

Product 3 share gained — captured most of Product 1's lost share

OBSERVATION

Cannibalization is failing. Product 1's share fell from 73.8% → 58.9%, but Product 2 dropped from 10.0% → 5.4% instead of absorbing it. Product 3 captured most of the shift, rising from 14.3% → 28.8% and roughly doubling its share.

RECOMMENDATIONS

■ Reposition Product 2 against Product 3, not Product 1

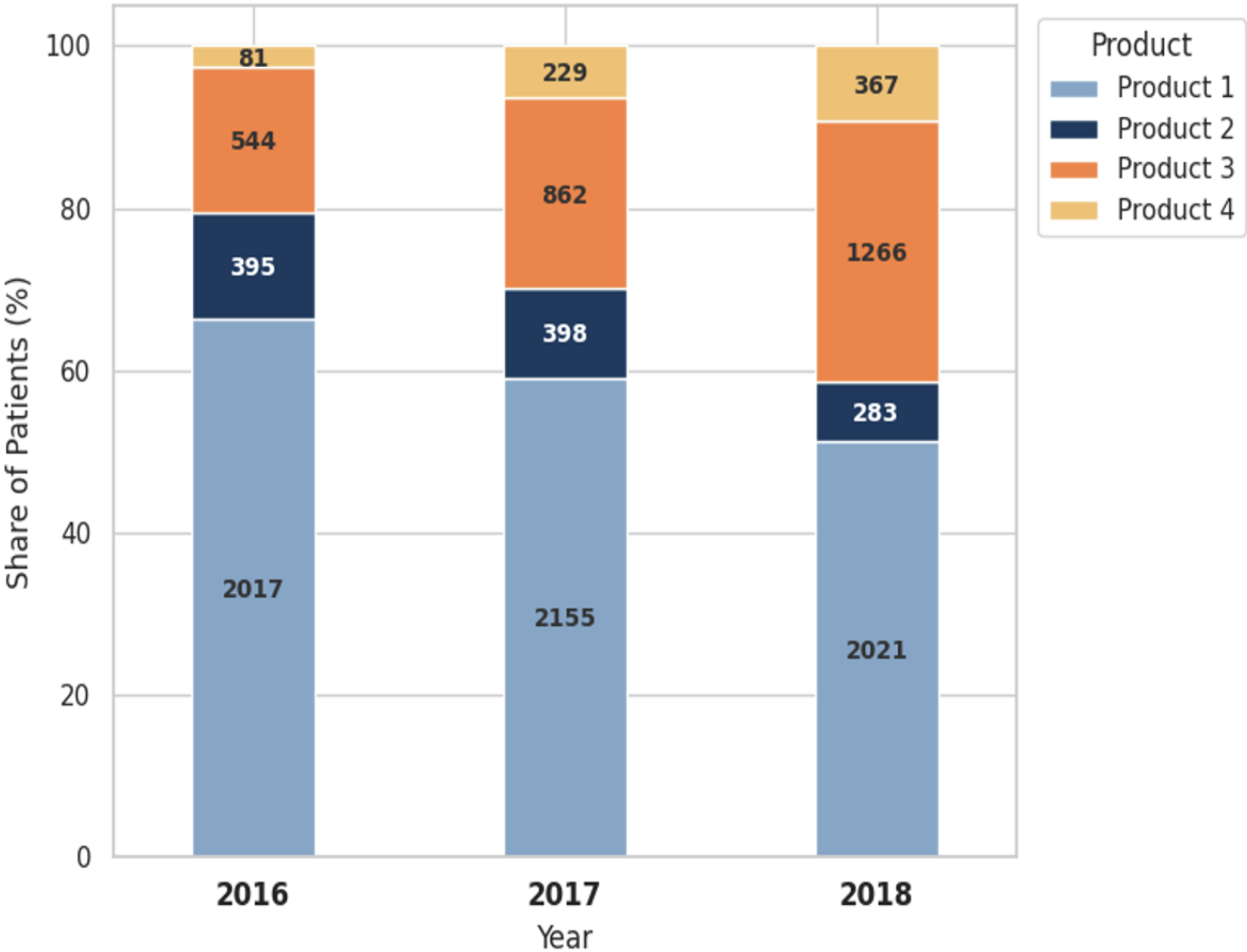
Position Product 2 as a differentiated alternative to Product 3, not just a portfolio replacement for Product 1. Move sales-force time share toward HCPs currently writing Product 1 + Product 3.

■ Match Product 3's promotional intensity in NPP & speaker programs

The 2017 → 2018 acceleration in Product 3's growth signals that competitor promotion is winning. Increase Product 2 NPP frequency, peer-led webinars, and clinical reprints through the same promotional channels Product 3 is winning in.

Share of Patients by Product (2016–2018)

Unique patient share in the injectable anesthesia market



-5.8 pts

Product 2 patient share — steady decline 2016 → 2018

+14.3 pts

Product 3 patient share — absorbed the shift

OBSERVATION

Product 2 isn't just losing claims — it's losing patients. Its patient share fell from 13.0% → 7.2%, while Product 3 nearly doubled from 17.9% → 32.2%. Product 2 is reaching fewer unique patients each year, not just fewer claims per patient.

RECOMMENDATIONS

■ **Patient-level engagement program**

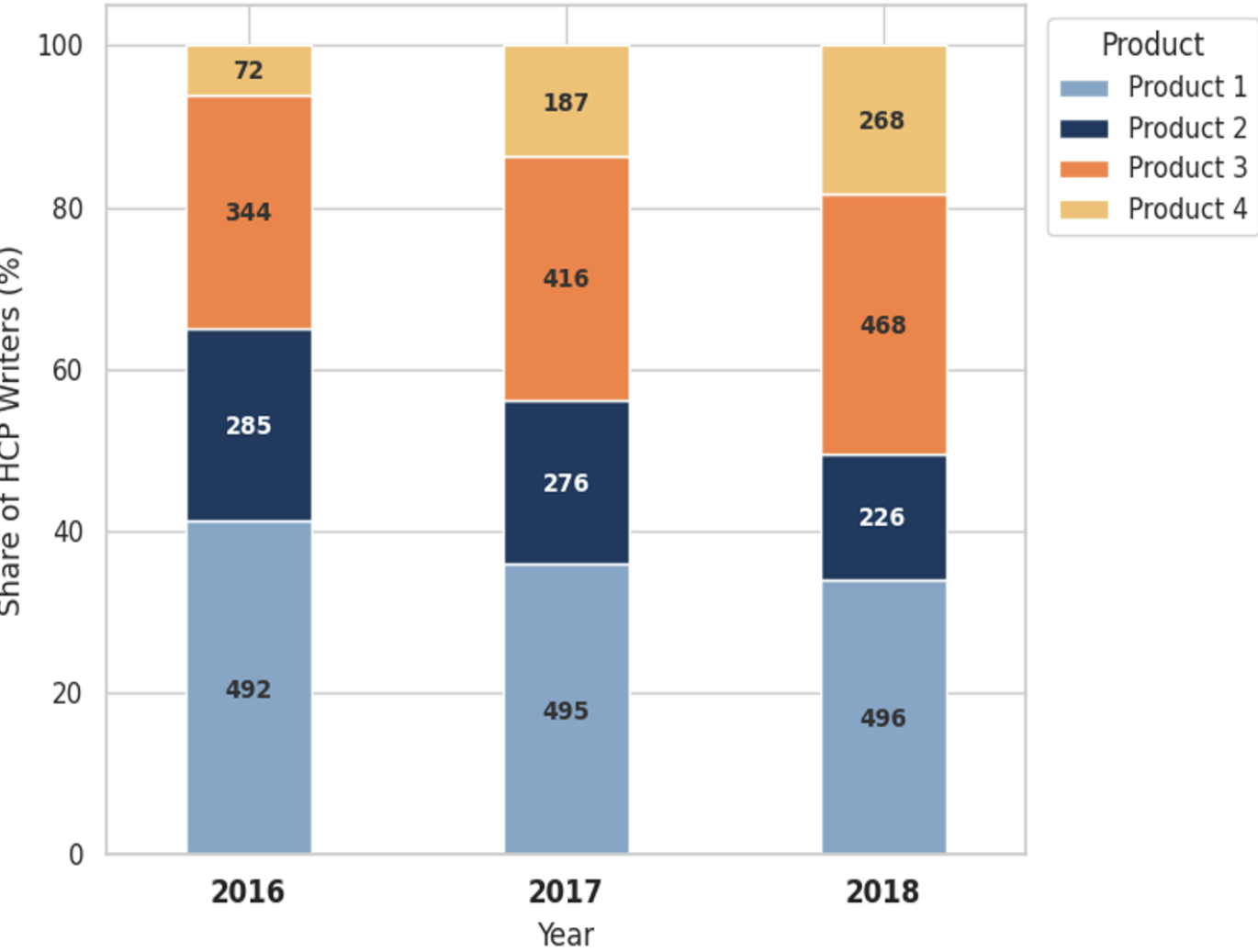
Patient counts are dropping faster than claim counts — patients aren't being retained on Product 2. Launch copay assistance, patient education, and adherence support to keep newly-acquired patients on therapy.

■ **Patient-mix audit**

Compare the patient profile (age, diagnosis, comorbidity) between Product 2 and Product 3 in the next phase of analysis to identify which patient segments Product 3 is capturing and why.

Share of HCPs (Writers) by Product (2016–2018)

Share of unique prescribers writing each product



-8.4 pts

Product 2 prescriber share — biggest writer drop in the basket

+12.4 pts

Product 4 prescriber share — quietly tripled (6.0% → 18.4%)

OBSERVATION

Product 2's writer share fell sharply from 23.9% → 15.5%; the absolute writer count dropped from 285 → 226. Product 1 held its writer base flat (~495), so the prescribers leaving Product 2 are mostly going to Product 3 and Product 4, not to our own leader brand.

RECOMMENDATIONS

■ **Prescriber retention is the priority**

Identify the ~60 HCPs lost between 2016–2018 and re-engage with field reps, samples, and tailored clinical content. Losing writers compounds — fewer writers means fewer patients next year.

■ **Watch Product 4 too**

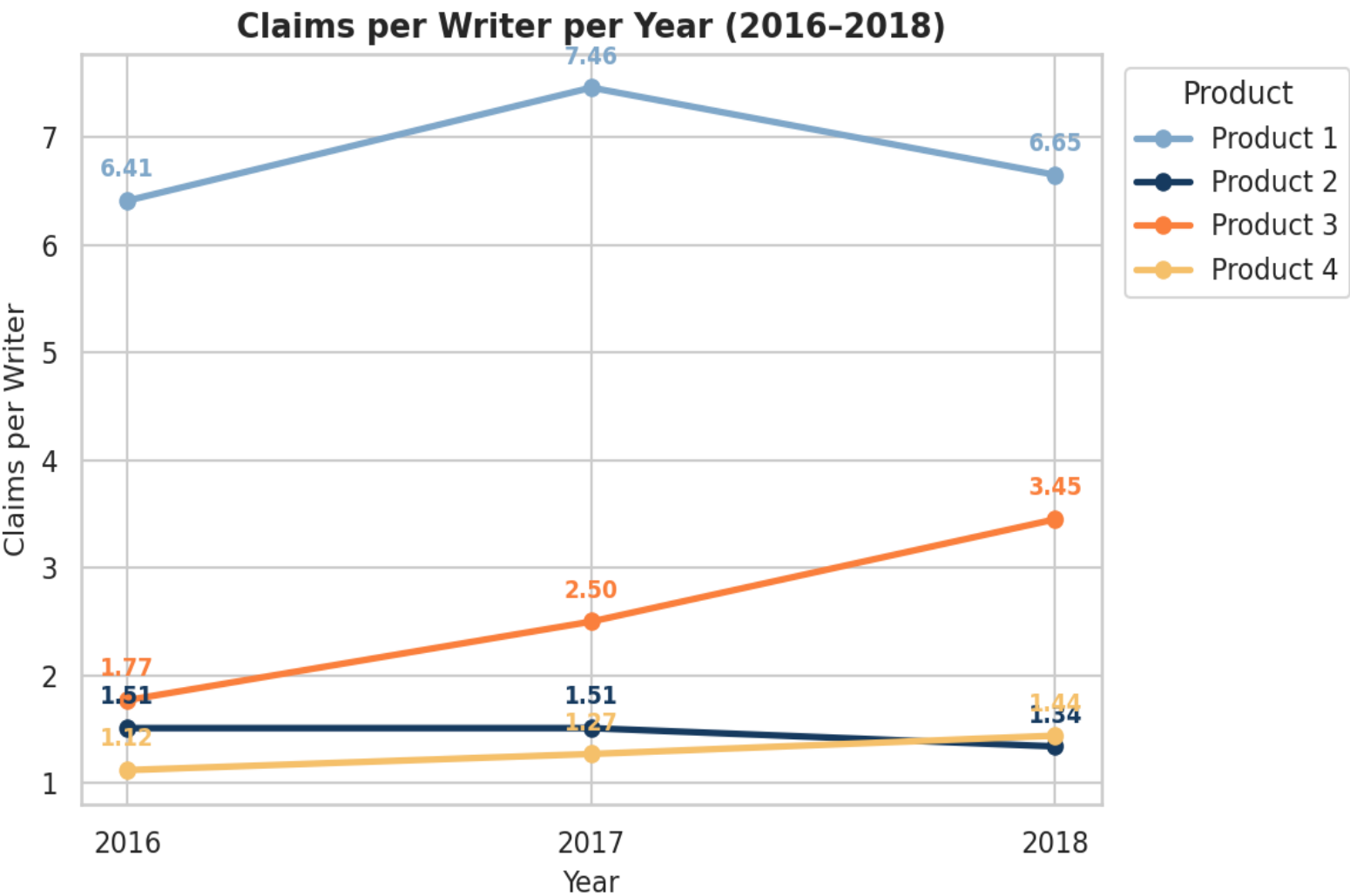
Product 4 added more new writers than any brand (72 → 268). Treat it as a rising competitive threat in subsequent analyses, not just a background player.

Q1 Part B & C (Writer Productivity + Territory Drop) – 4 Charts

Bhuvan's Work (Slide 13-16)



KBQ 1B · Claims per Writer per Year (2016–2018)



[← Back to Priority Matrix](#)

3.45 P3 2018 Claims/Writer	1.34 P2 2018 Claims/Writer
+95% P3 Growth 2016→2018	-11% P2 Change 2016→2018

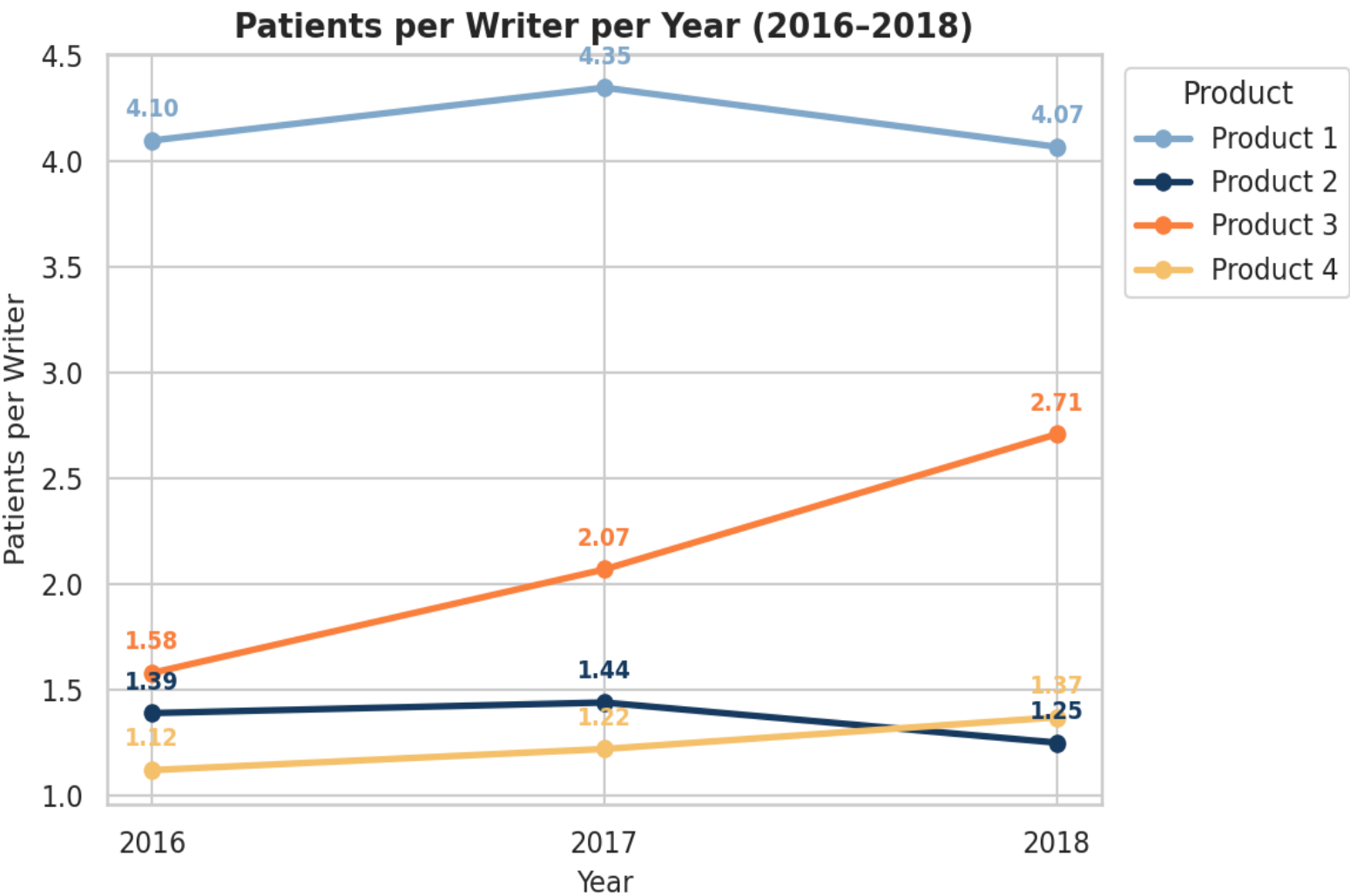
Observation

Product 3 (Fentirate) nearly doubled prescribing intensity — from 1.77 to 3.45 claims per writer. Product 2 (Midoride) stayed flat then dropped to 1.34: even loyal writers are prescribing it less frequently.

Recommendation

Give existing Midoride writers clinical evidence kits and detailing tools. Target highest-volume Product 1 writers (6.65 claims/writer) as prime conversion candidates for Midoride.

KBQ 1B · Patients per Writer per Year (2016–2018)



2.71

P3 2018
Pats/Writer

1.25

P2 2018
Pats/Writer

+71%

P3 Patient
Reach Growth

-10%

P2 Patient
Reach Change

Observation

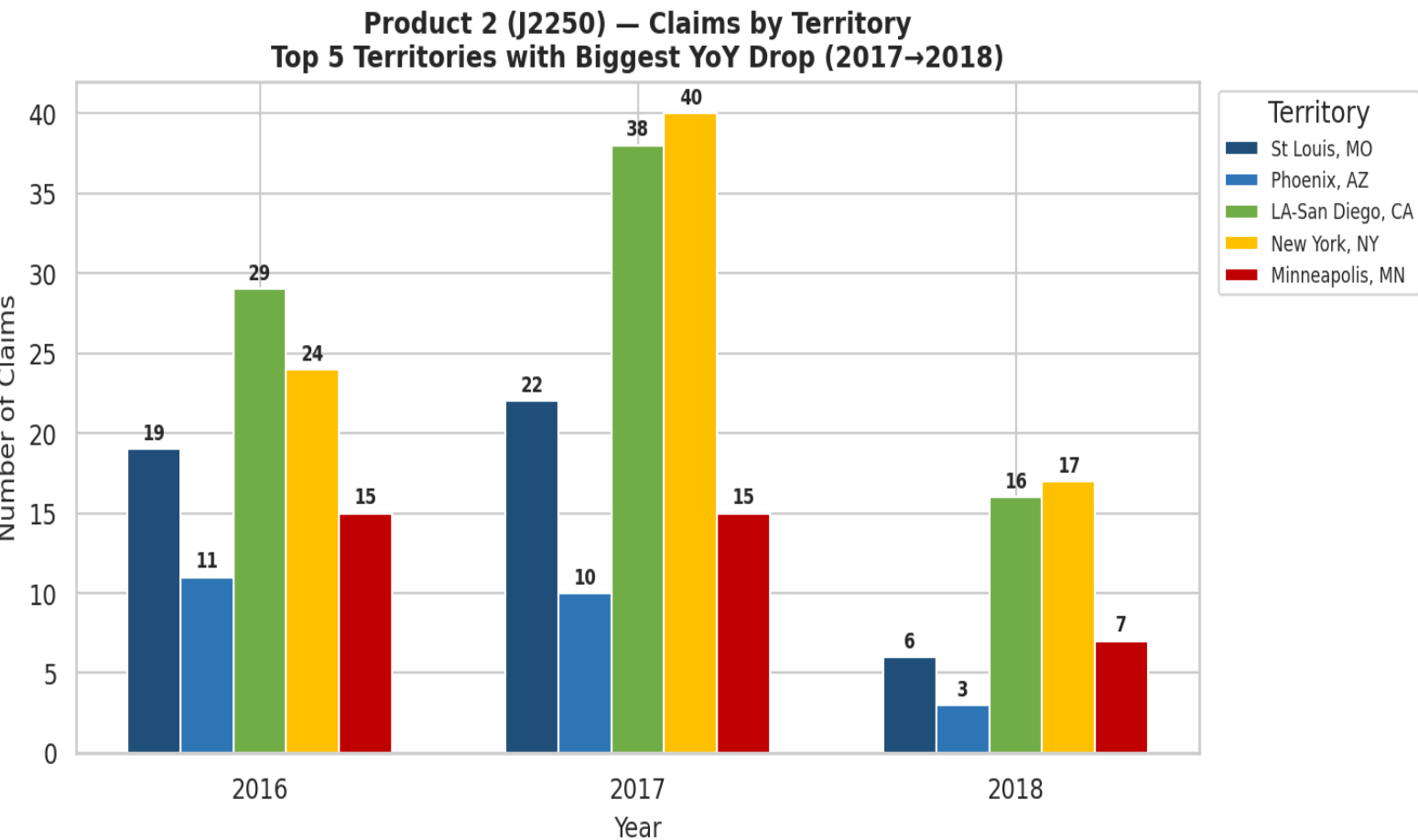
Fentirate's patient reach per writer grew from 1.58 to 2.71 — a 71% increase. Midoride fell from 1.39 to 1.25. Writers aren't just prescribing Midoride less — they are treating fewer unique patients with it each year.

Recommendation

Launch copay assistance and patient support services for Midoride writers to lower the barrier to initiating new patients. Prioritize writers whose patient count dropped YoY for urgent rep visits.

[← Back to Priority Matrix](#)

KBQ 1C · Product 2 (J2250) — Claims in Top 5 Declining Territories



YoY Drop: 2017 → 2018			
Territory	2017	2018	Change
St. Louis, MO	22	6	-73%
Phoenix, AZ	10	3	-70%
LA-San Diego, CA	38	16	-58%
New York, NY	40	17	-58%
Minneapolis, MN	15	7	-53%

Observation

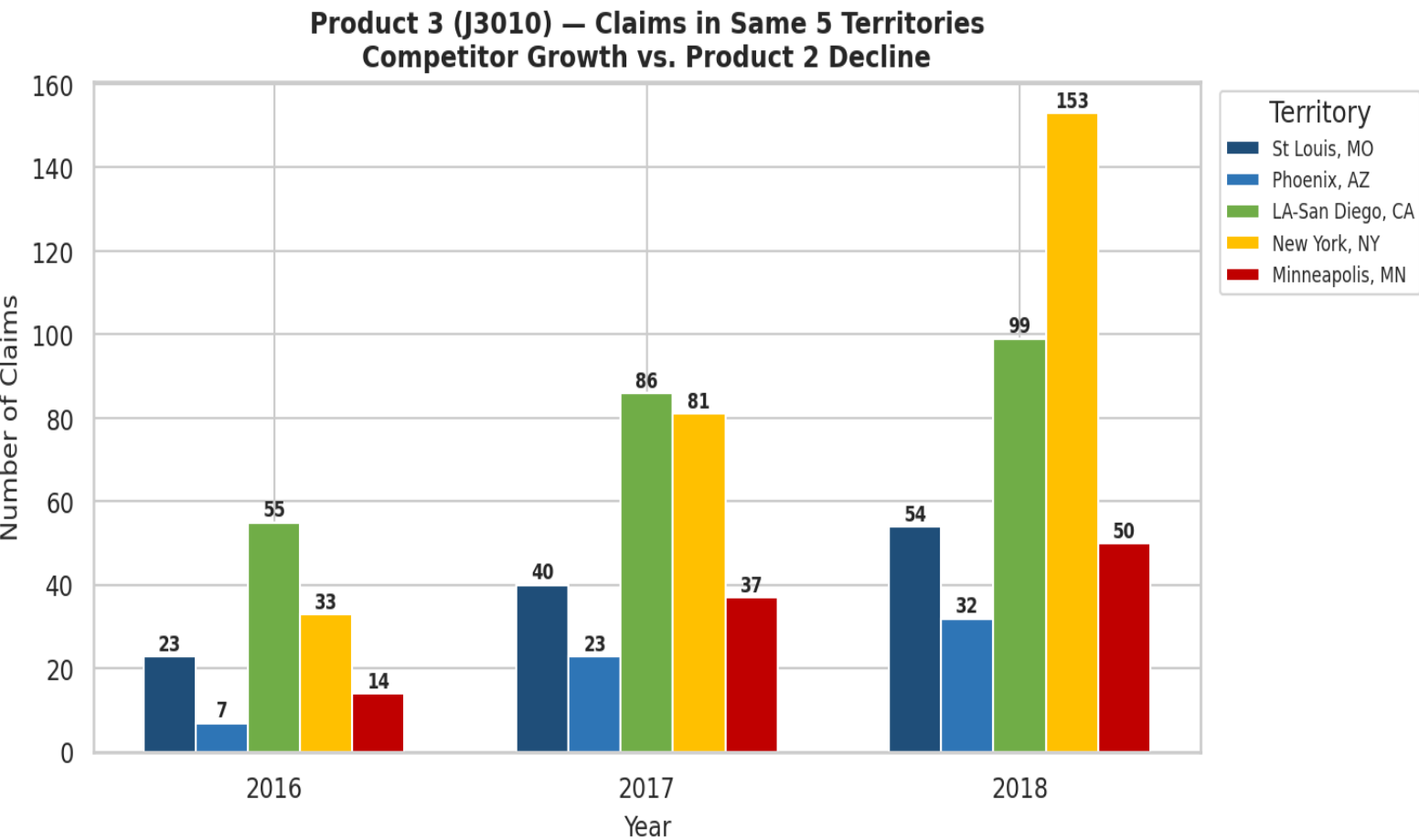
LA-San Diego and New York had the highest absolute claim volumes — their ~58% drops are the biggest revenue losses. St. Louis (-73%) and Phoenix (-70%) show steeper % drops — near-complete competitive displacement in smaller markets.

Recommendation

HIGH VOLUME (LA, NY): Redeploy field reps immediately — even recovering 20% of lost claims has massive ROI. LOW VOLUME (Phoenix, St. Louis): Deploy cost-efficient digital NPP campaigns instead of expensive field redeployment.

[← Back to Priority Matrix](#)

KBQ 1C · Product 3 (J3010) — Competitor Growth in Same 5 Territories



Fentirate Growth: 2016 → 2018			
Territory	2016	2018	Growth
New York, NY	33	153	+364%
Phoenix, AZ	7	32	+357%
Minneapolis, MN	14	50	+257%
St. Louis, MO	23	54	+135%
LA-San Diego, CA	55	99	+80%

Observation

Fentirate is surging in every territory where Midoride is collapsing. New York: +364% while Midoride fell -58%. This is NOT market contraction — it is direct, territory-by-territory competitive cannibalization by Product 3.

Recommendation

Run HCP-level win-back analysis in New York and LA to identify which specific writers switched from Midoride to Fentirate. Reach those writers urgently with tailored clinical messaging and Midoride trial incentives.

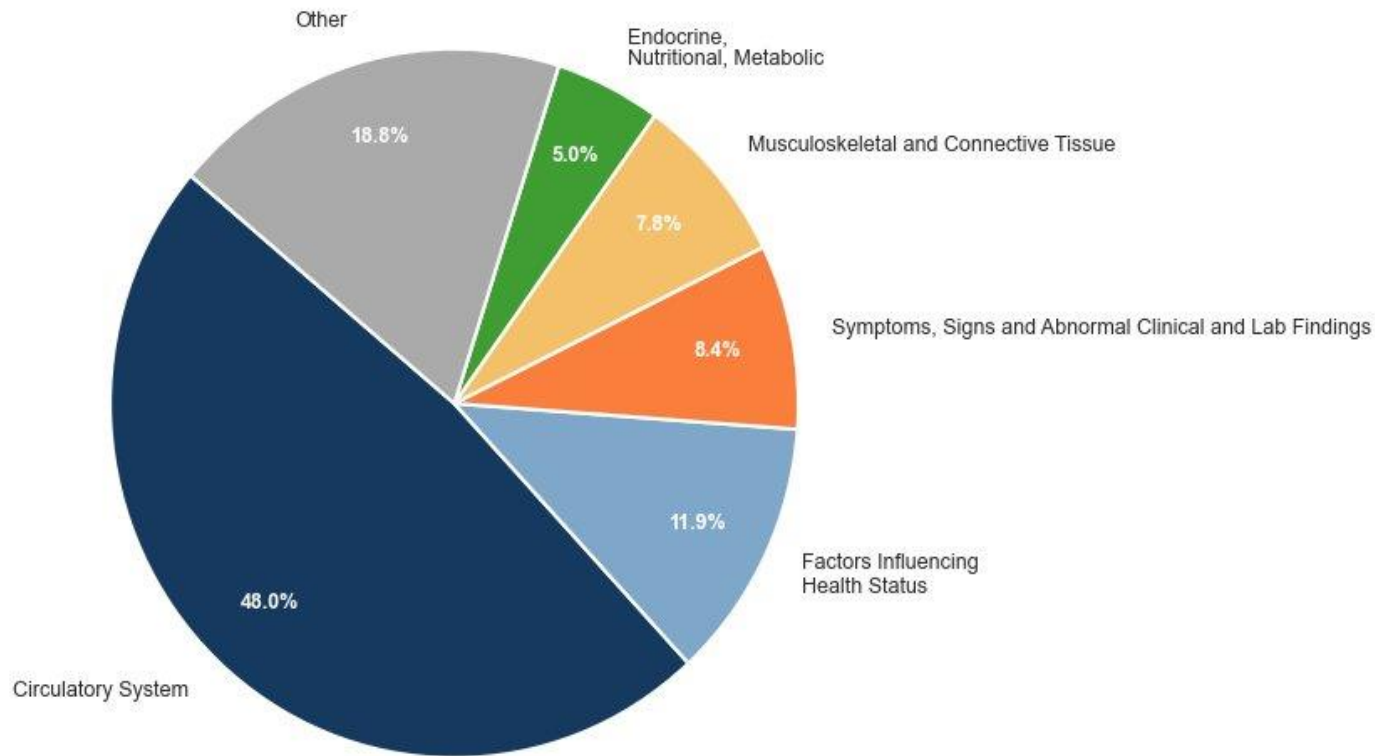
An analysis of Diagnosis, Specialty, Age & Writer Trends



Top 5 Diagnosis Specialties – Share of Claims (2016–2018)

All 4 Brands combined · ICD-10 diagnosis codes mapped to specialty

Top 5 Diagnosis Specialties — Share of Claims
(All 4 Brands, 2016–2018)



48%

Circulatory System
#1 Diagnosis Specialty

54%

Top 2 Specialties
Circulatory + Factors

Diagnosis Specialty

Circulatory System	48.0%
Factors Influencing Health	11.9%
Symptoms & Signs	8.4%
Musculoskeletal	7.8%
Endocrine, Nutritional	5.0%
Other	18.8%

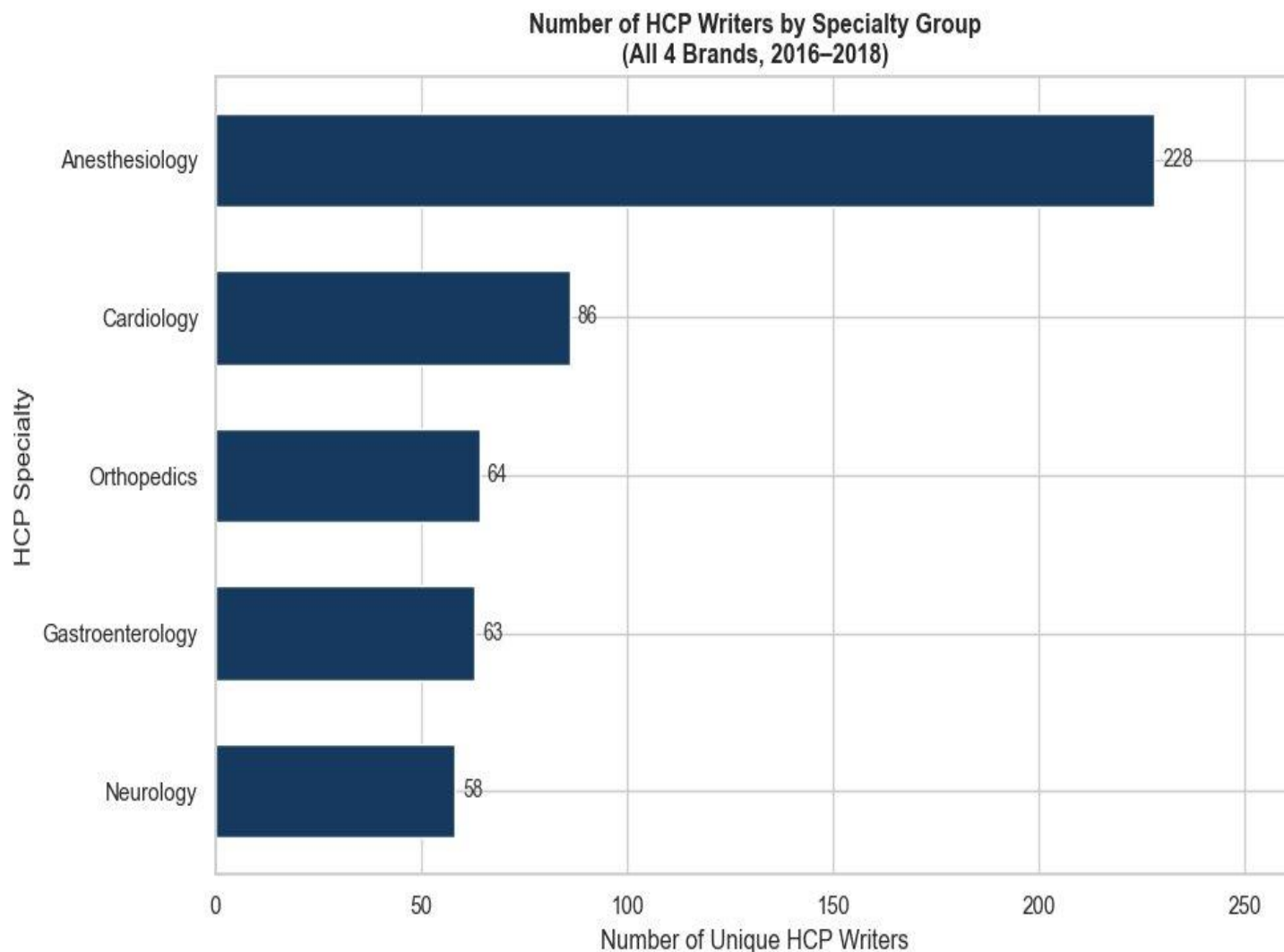
Observation

Circulatory System dominates at 48% — 4x the next group. That means cardiovascular patients are the backbone of this market, and Product 2 needs to be in front of the Cardiologists treating them, not just Anesthesiologists.



HCP Writers by Specialty Group (2016–2018)

All 4 Brands combined · Unique HCP writers deduplicated across 2016–2018



228

Anesthesiology
#1 Writer Specialty

3x

Anesth. vs Cardiology
228 vs 86 writers

HCP Specialty

Writers

Anesthesiology - 228
Cardiology - 86
Orthopedics - 64
Gastroenterology - 63
Neurology - 58



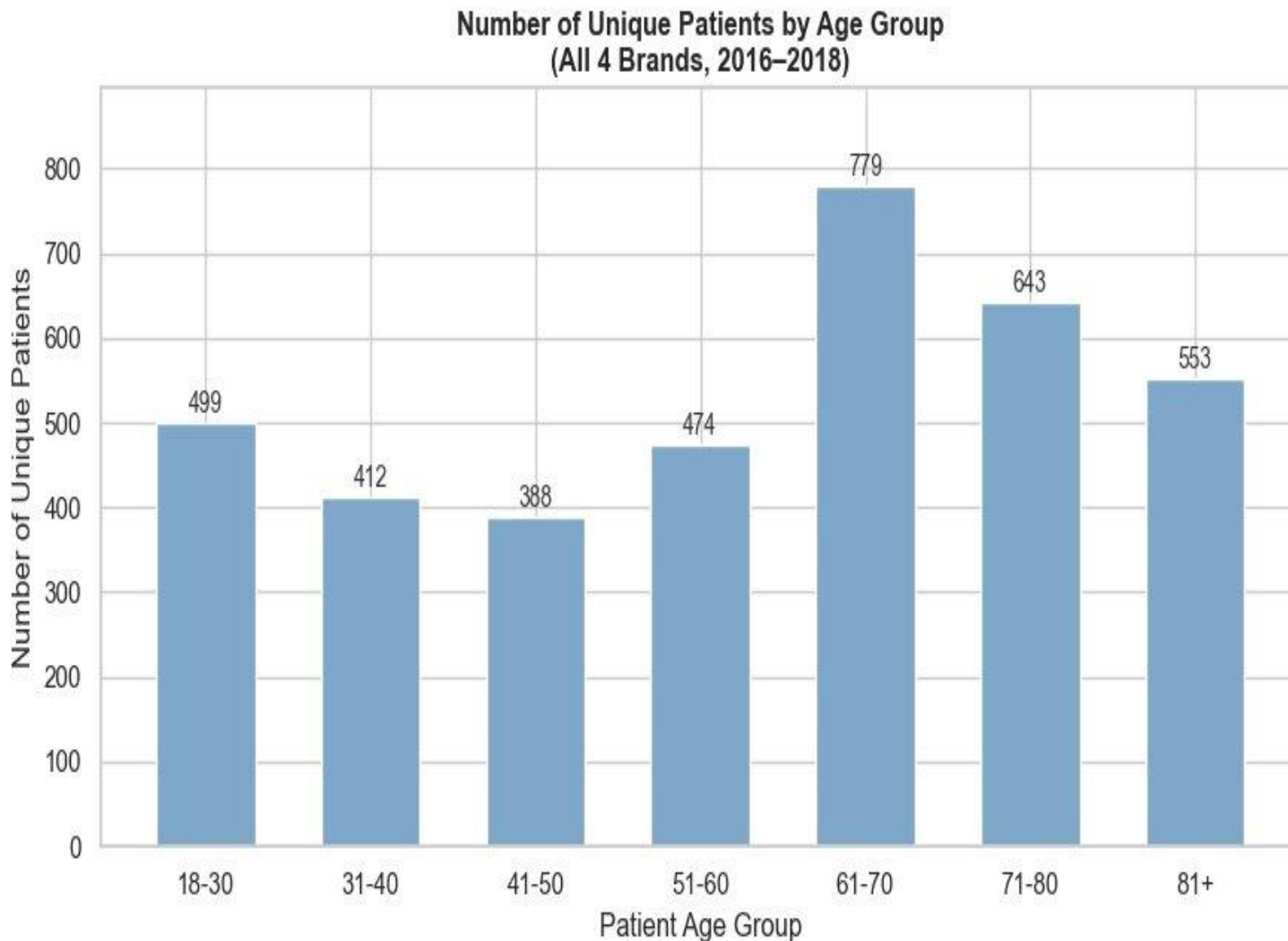
Observation

- Anesthesiology leads with 228 writers — nearly 3x Cardiology (86), the #1 priority for Product 2.
- Product 3 is gaining uniformly across all 5 specialties, ruling out a targeting problem.
- A broad multi-specialty strategy is essential — narrowing to Anesthesiology alone will not reverse the decline.



Unique Patients by Age Group (2016–2018)

All 4 Brands combined · Medicare patient population 2016–2018



779

Patients aged 61–70
Largest age group

54%+

Patients aged 61+
of total patient base

Age Group

Patients

18–30 --> 499

31–40 --> 412

41–50 --> 388

51–60 --> 474

61–70 --> 779 ▲

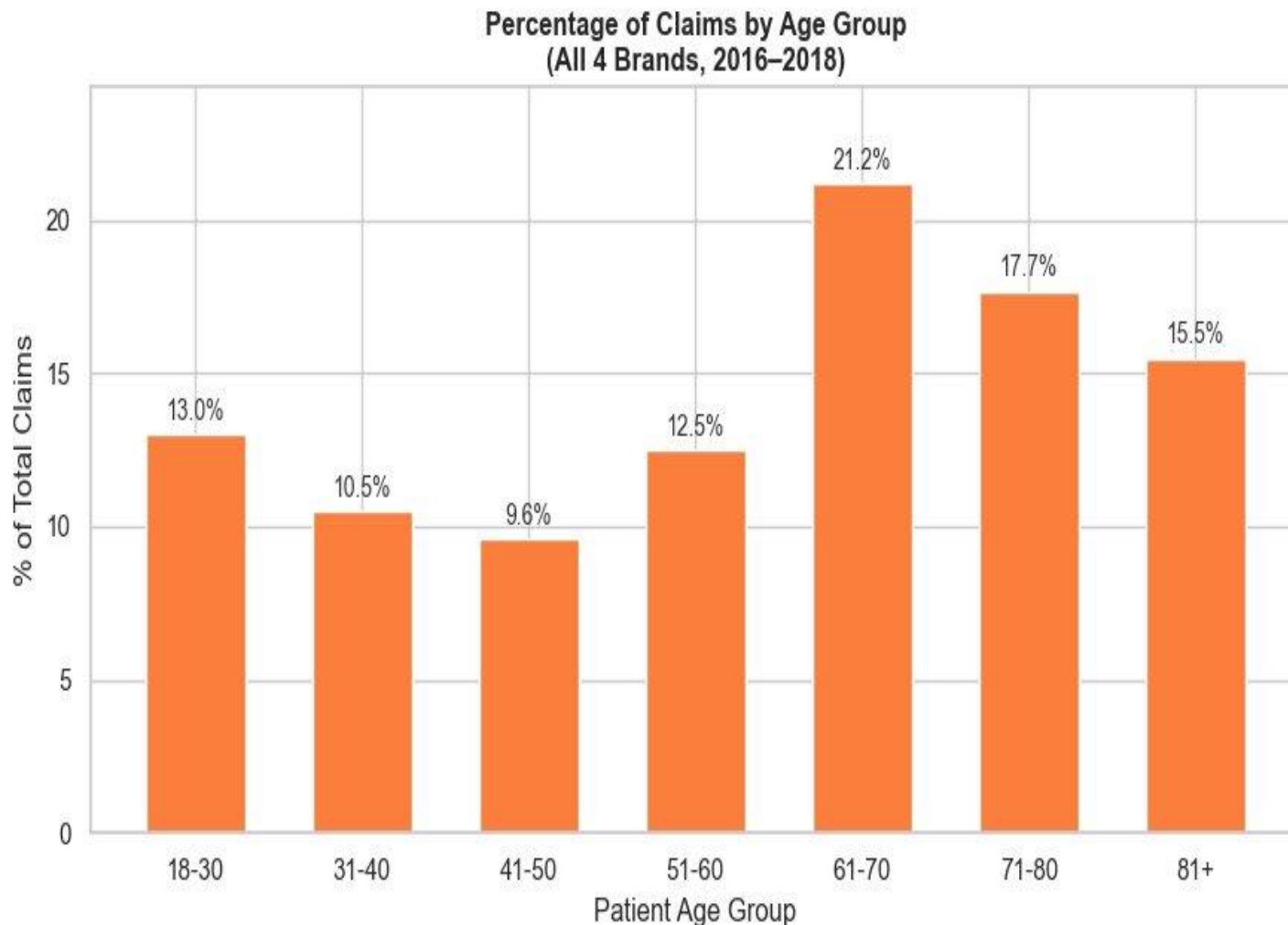
71–80 --> 643

81+ --> 553



Percentage of Claims by Age Group (2016–2018)

All 4 Brands combined · % of total claims per age bucket



21.2%

Claims aged 61–70
Highest claim share

54.4%

Claims from 61+
Medicare core segment

Age Group

% Claims

18–30 --> 13.0%

31–40 --> 10.5%

41–50 --> 9.6%

51–60 --> 12.5%

61–70 --> 21.2% ▲

71–80 --> 17.7%

81+--> 15.5%

Diagnosis Specialty · HCP Writers · Patient Age

OBSERVATION

RECOMMENDATION 1

61–70 Cohort Priority

- The 61–70 cohort is the highest volume segment and should be the first priority for Product 2 detailing and patient support programs.
- Clinical messaging should emphasize Product 2's safety profile and dosing guidelines specifically for elderly surgical patients.
- HCPs treating high volumes of 61–70 patients such as general surgeons and anesthesiologists should be identified and targeted first.

RECOMMENDATION 2

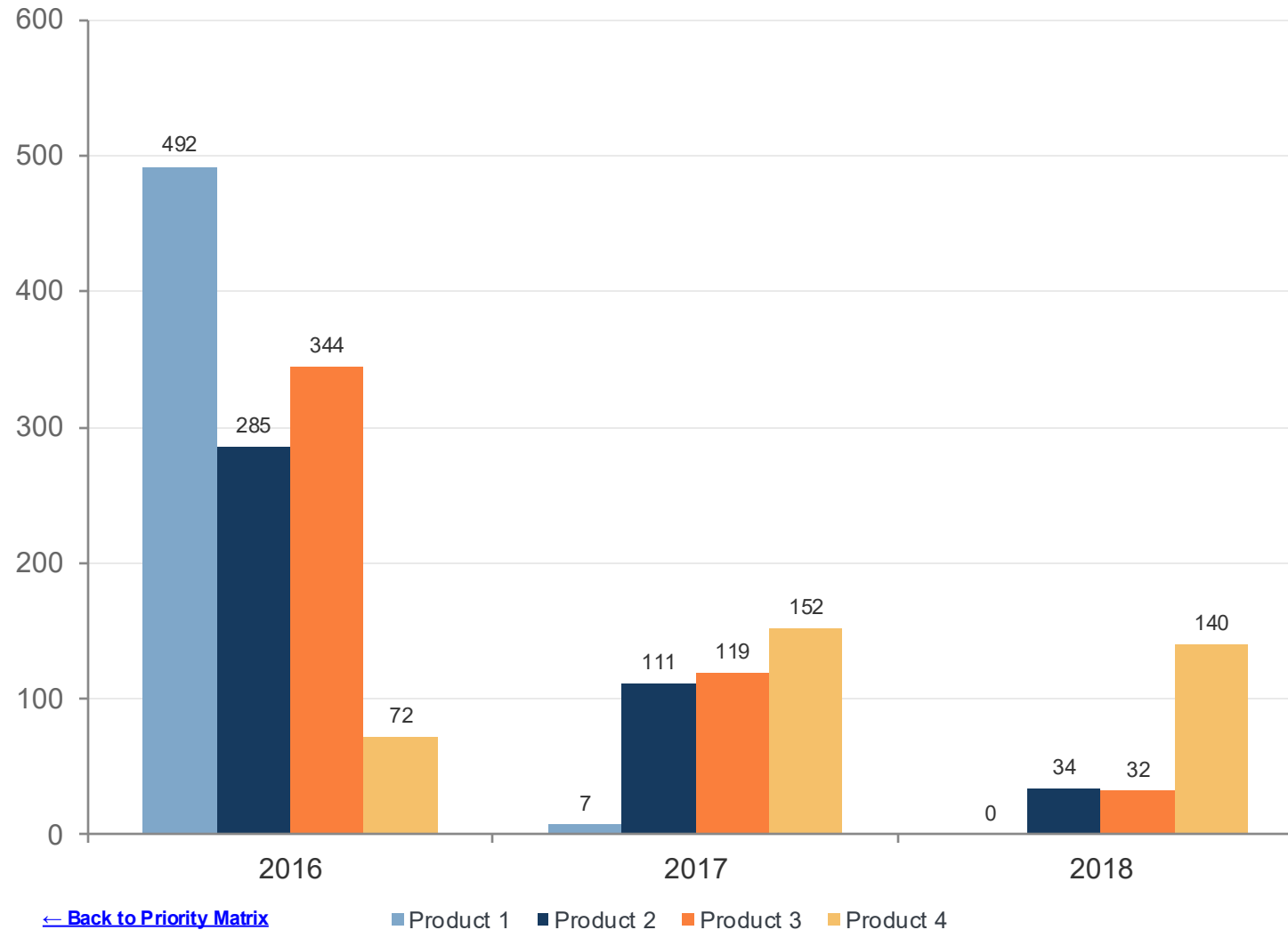
Broad Age Strategy:

- Since Product 3 is eroding Product 2's share equally across all age groups, age based targeting alone will not reverse the decline.
- A market wide engagement strategy must run in parallel with any age specific efforts to address the systemic competitive threat.
- Sales force coverage should be evaluated across all age cohorts to ensure no segment is being under-served relative to Product 3's reach.



New HCP Writers per Brand per Year (2016–2018)

Q2 Part B · Writer Acquisition Trends



-69%

Product 2 new writers
2017→2018 (111→34)

140

Product 4 new writers
in 2018 — highest!

0

Product 1 new writers
in 2018 (saturated)



OBSERVATION

Product 2's new writer acquisition collapsed from 111 → 34 (-69%) in 2017→2018, nearly identical to Product 3 (119→32). However, Product 3 compensates through a far larger continuing writer base. Most critically, Product 4 attracted 140 new writers in 2018 — highest across all brands — confirming the total market writer base is still growing (1,193→1,458). Product 2's collapse is competitive displacement, not market saturation.

01

Target Product 4's 140 New 2018 Writers

REC

These HCPs are actively entering the market but bypassing Product 2. Cross-reference Q2 Part A HCP specialty data to deploy targeted detailing in those exact specialties before prescribing habits solidify.

02

Link New-Writer Collapse to Top-5 Drop Territories

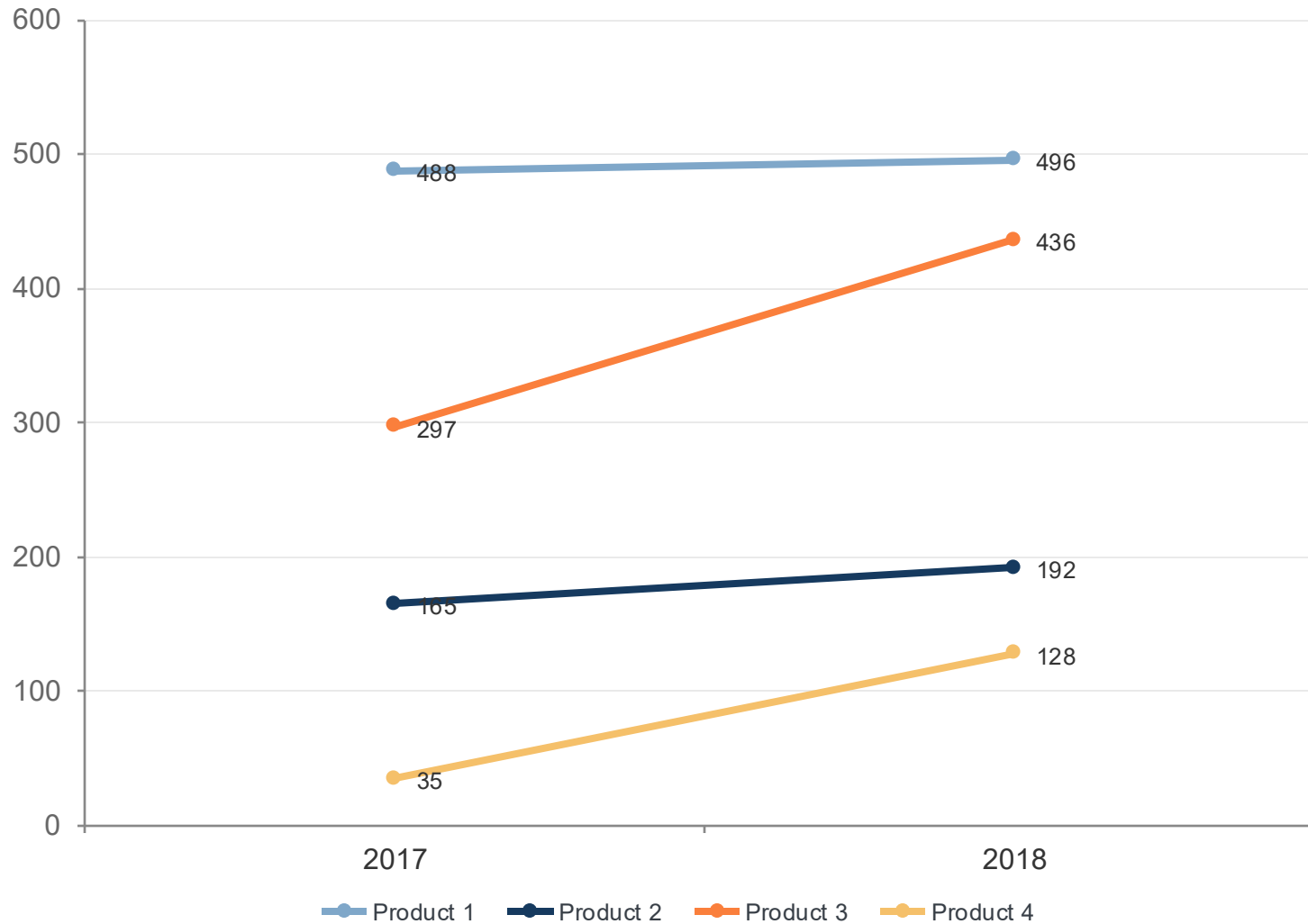
REC

If the 2017→2018 new-writer decline is geographically concentrated in the top 5 dropping territories (St Louis, Phoenix, NY, LA, Minneapolis), a focused field reallocation will recover new-writer acquisition more efficiently than a national campaign.



Continuing HCP Writers per Brand (2017–2018)

Q2 Part B - Retention Trends



+47%

Product 3 continuing
writers 2017→2018
(297 → 436)

+16%

Product 2 continuing
writers 2017→2018
(165 → 192)

×2.3

Product 3 base vs
Product 2 in 2018
(436 vs 192)



Q2 Part B

OBSERVATION

Product 3's continuing writer base grew 297→436 (+47%) vs Product 2's 165→192 (+16%) — less than half of Product 3's absolute base by 2018 and the gap is widening. Even Product 1 holds a stable continuing base of 488→496, confirming its decline is reduced prescribing frequency, not writer attrition. Product 2 faces a dual problem: small continuing base AND weak per-writer prescribing frequency.

01

Defend the 192 Continuing Writers — Highest ROI

REC

Per Q1 Part B claims-per-writer data, these writers are prescribing less frequently. Use CRM to identify high-decile vs low-decile continuing writers and deploy prioritized field engagement to increase prescription intensity before they shift to Product 3.

02

Contest Specialties Driving Product 3 Retention

REC

Product 3's retention advantage (297→436) likely reflects stronger formulary access or specialty alignment. Cross-reference Q2 Part A HCP specialty chart — if Product 3 is over-indexed in highest-volume specialties, reallocate Product 2 field & NPP resources to contest those specialties.



Strategic Recommendations

Stopping Market Share Erosion for Product 2



Q3 Priority Matrix — Analytics-Backed Actions

Immediate Actions + Strategic Initiatives

PRIORITY	ACTION	SUPPORTING ANALYSIS
CRITICAL	Territory Win-Back — Top 5 Geographies	Q1 Part C — Clustered Bar Charts (Prod 2 vs Prod 3)
CRITICAL	Co-Pay / Payer Assistance — Access Friction Fix	Q1 Part B — Patients per Writer + Q1 Part A — Share of Patients
HIGH	Targeted NPP Digital — New Writer Acquisition	Q1 Part A — HCP Writer Share + Q2 Part B — New Writers Chart
MEDIUM	Brand Repositioning — Claims per Writer Growth	Q1 Part B — Claims per Writer Line Graph
MEDIUM	HCP Multi-Specialty Engagement Program	Q2 Part A — HCP Specialty Bar + Diagnosis Specialty Pie
MEDIUM	Patient Segment Focus — 61–70 Age Cohort	Q2 Part A — Age Bar Graphs + Q1 Part A — Share of Patients



Immediate Action 1 — Territory Win-Back

CRITICAL · Backed by Q1 Part C Territory Charts

Product 2's 2018 collapse was a discrete, geographically concentrated displacement event. In 5 top-drop territories, Product 2 lost 53–73% of claims in a single year while Product 3 grew in every territory simultaneously.

TERRITORY	PROD 2 (2017→2018)	YoY DROP	PROD 3 (2017→2018)
St Louis, MO	22→6	-72.7%	40→54
Phoenix, AZ	10→3	-70.0%	23→32
LA–San Diego, CA	38→16	-57.9%	86→99
New York, NY	40→17	-57.5%	81→153 ⚡
Minneapolis, MN	15→7	-53.3%	37→50

- Deploy dedicated account executives immediately to all 5 territories over the next 1-2 sales cycles
- Metros (NY, LA, St Louis): broad field coverage + peer-to-peer speaker programs at academic centers
- Mid-size (Phoenix, Minneapolis): one-to-one key account management to defend each prescriber relationship



Immediate Actions 2 & 3 — Access + Digital Promotion

CRITICAL · Backed by Q1 Part B, Q1 Part A, Q2 Part B

02 CO-PAY / PAYER ASSISTANCE

Signal: 3 charts converge on the same finding — Product 2 is losing patients faster than doctors.

- Patients/writer: 1.39→1.25 (-10.1%) while Product 3 grew 1.58→2.71 (+71.5%)
- Product 2 unique patients fell 395→283 as total market grew 3,037→3,937
- Classic formulary access / cost-sharing friction pattern

ACTIONS:

- Expand co-pay assistance & communicate to 192 continuing writers
- Audit Product 2 formulary tier in the 5 drop territories
- Track prescription → dispensed abandonment rates

03 TARGETED NPP DIGITAL

Signal: Market writer base grew 1,193→1,458, yet Product 2's writer count fell 285→226.

- Product 2 attracted only 34 new writers in 2018 vs Product 4's 140
- New HCPs entering market are systematically bypassing Product 2
- Anesthesiology: 228 writers — 3× next specialty (Cardiology: 86)

ACTIONS:

- Target HCPs who haven't yet prescribed any market basket brand — highest conversion potential
- Specialty-filtered digital campaigns: Anesthesiology, Cardiology, Orthopedics, Gastroenterology
- Geo-targeted digital + in-person rep visits in the 5 drop territories simultaneously



Strategic Initiatives — Brand & HCP Engagement (6–18 Months)

MEDIUM Priority · Q1 Part B & Q2 Part A

04 BRAND REPOSITIONING

Claims per Writer Growth

Product 3's per-writer productivity nearly doubled (1.77→3.45, +94.9%) while Product 2 declined (1.51→1.34, -11.3%). Product 2 doctors are narrowing the patient cohort per prescriber — a clinical confidence problem.

ACTIONS:

- Develop case-based selling materials for specific patient profiles & procedural contexts
- Build KOL speaker program from top-decile Product 2 writers (Anesthesiology priority)
- Train reps to lead with the productivity narrative vs Product 3

05 HCP SPECIALTY ENGAGEMENT

Multi-Specialty Program

Anesthesiology: 228 writers (dominant). Cardiology: 86 writers treating the 48% Circulatory System claim base — the highest-volume diagnosis. Cardiology is likely under-penetrated relative to its claim potential.

ACTIONS:

- Specialty-specific promotional programs for all top 5 HCP groups
- Cardiology messaging: connect Product 2 to the 48% Circulatory patient base
- Specialty-specific speaker bureaus — KOLs from each group (Anesthesiology ≠ Orthopedics context)

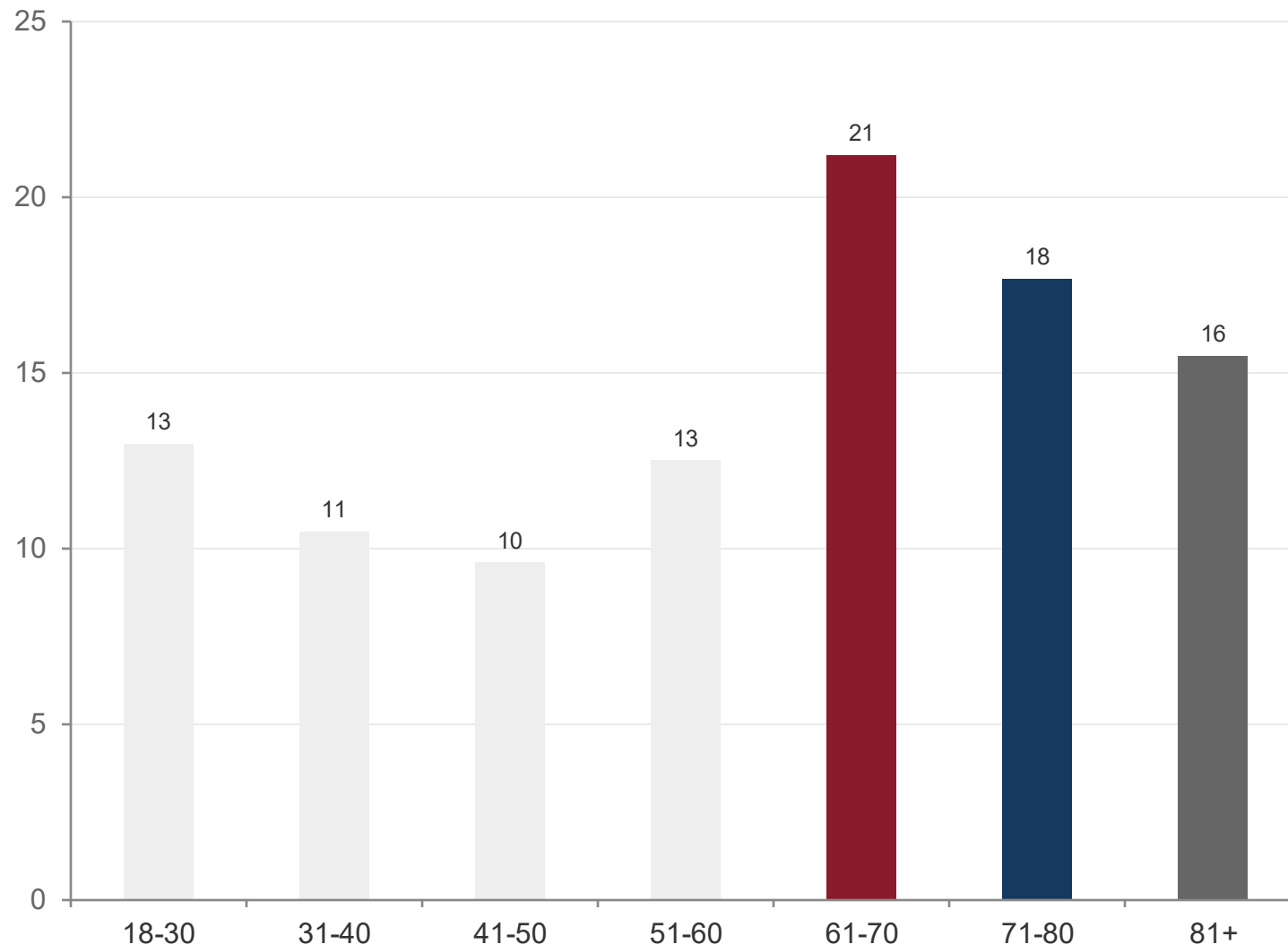


Strategic Initiative 6 — Patient Segment Focus (61–70 Cohort)

STEVENS

INSTITUTE of TECHNOLOGY

MEDIUM Priority · Q2 Part A Age Distribution



KEY INSIGHT

Patients 61+ account for >54% of all claims. Yet Product 2's patient base fell 395→283 while total market grew 3,037→3,937. Product 3 captured these patients: 544→1,266 unique patients.

54%+

Claims from
patients 61+

779

Unique patients
61-70 (highest)

-29%

Product 2 patient
base 2016→2018

- Build 61–70 patient support programs: adherence tools, nursing hotlines, caregiver resources
- Clinical messaging: Product 2 safety/dosing for elderly surgical patients vs J3010 (fentanyl)
- Monitor 51–60 cohort (12.5% claims) as emerging acquisition opportunity

5 Key Takeaways

Conclusion — Team 2, BIA 810 Spring 2026

1

Product 3 captured Product 1's share, not Product 2

Product 1 lost 14.9 pts of claim share; Product 3 gained 14.5 pts. Product 2 shrunk 4.6 pts instead of growing. This is failed cannibalization — Product 2 needs urgent repositioning.

2

The erosion is geographically concentrated

5 territories account for the steepest 2018 drops (53–73%) for Product 2, while Product 3 grew in all 5 simultaneously. Territory win-back is the single highest-ROI immediate action.

3

New writer acquisition has collapsed — but the market is NOT saturated

Product 4 attracted 140 new writers in 2018. The market is growing. Product 2's new-writer failure is competitive displacement.

4

Continuing writer retention is Product 2's lifeline

192 continuing writers in 2018. These prescribers are prescribing less frequently. Defending and deepening this base is the highest-ROI retention action available.

5

Multi-specialty + multi-segment strategy required

Product 3 is gaining uniformly across all specialties and age groups. A narrow focus on Anesthesiology or the 61–70 cohort alone will not reverse the trend — a coordinated broad strategy is essential.



Data Exploration Opportunities

What would sharpen this analysis?



Data Gap – Missing from Claims

Critical fields absent from the given dataset that limit analytical depth

Quantity & Dosage

- Claim counts are declining (documented in KBQ1), but without quantity dispensed per claim, **total drug volume** cannot be calculated
- Volume decline may be steeper or shallower than claim count trends alone suggest
- With dosage data: prescribing habits by care setting and specialty can be analyzed, informing **packaging format** decisions that improve administration convenience

Patient Comorbidity Depth

- Current analysis maps only the first character of one diagnosis code per claim; secondary diagnoses and full ICD-10 detail are not captured
- Cannot assess whether Product 2 and Product 3 are being used in clinically distinct patient populations
- With full comorbidity profiles: determine whether Product 3's growth is driven by clinical protocol preference rather than promotional or formulary factors

Reimbursement & Cost

- No allowed amounts, net reimbursement, or place-of-service codes in current data
- Cannot assess whether Product 2 carries a pricing disadvantage under Medicare Part B ASP framework
- Margin differential across care settings (inpatient, ASC, physician office) is a primary input for P&T Committee and formulary tier decisions



Data Gap – External Commercial Datasets

Adatasets from commercial pharmacy analytics that would sharpen the diagnosis

Xponent / NPA

- Current analysis covers Medicare only; commercial insurance and Medicaid are excluded
- Cannot determine whether competitive displacement by Product 3 is Medicare-specific or market-wide
- Full-payer prescription data required for accurate market share calculation and payer-level strategy prioritization

Calls Data (CRM)

- No visibility into field rep engagement with HCPs in the five drop territories
- Cannot isolate whether the prescribing collapse was preceded by reduced field coverage or driven by competitive and formulary factors
- Territory-level call frequency vs. prescribing trend comparison would provide a diagnostic framework for rep reallocation

NPP Data

- No data on promotional reach, channel mix, or HCP response to non-personal promotion
- Cannot assess whether investment is reaching Anesthesiology writers or the new writer cohort that collapsed from 111 to 34 in 2018
- Cross-referencing NPP touchpoints with new writer trends would identify which channels and geographies drive HCP conversion



Is Product 2's erosion structural or addressable through targeted intervention?

1

Formulary Positioning

Reimbursement data needed to determine whether Product 3 holds a pricing or tier advantage at key hospital systems in drop territories; if confirmed, pharmacoeconomic evidence submission to P&T Committees is required

2

Clinical Protocol

Comorbidity depth analysis is needed to establish whether displacement in high-complexity patient segments reflects clinical protocol preference; if so, access response must extend beyond formulary to include medical affairs engagement and clinical evidence submission

3

Payer Scope

Xponent data needed to determine whether displacement is Medicare-only or extends across commercial and Medicaid; determines scale of access intervention required

4

Commercial Engagement

Calls data needed to establish whether field coverage gaps in priority territories preceded the prescribing decline; NPP data needed to confirm whether promotional messaging is reaching the right prescriber segments



Is volume loss driven by prescriber preference, administration factors, or clinical protocol?

1

Quantity / Dosage

Total drug volume cannot be calculated from claim counts alone; with dosage data, prescribing habits by setting and specialty can be analyzed to inform packaging formats that reduce administration burden and improve convenience

2

Patient Comorbidity Depth

If Product 3's growth is concentrated in higher-comorbidity patients, displacement reflects a clinical protocol preference; product-level response requires clinical evidence generation and indication or protocol positioning targeting those specific patient profiles

3

Reimbursement Cost

Economic return to the administering facility differs by care setting. Even with formulary access, facilities may prefer Product 3 if it generates better margin. Site-differentiated contract pricing may be the strategy to win back those high-risk territories



Q&A





Thank you!